



Carousel Fertility

*The fertility companion patients trust —
and the platform clinics need.*

FOUNDER

Nearly 20-year operator at Talkspace, SonderMind, Amazon, and deep personal experience with her own egg freezing journey make Sarah uniquely right to lead.



FOUNDER & CEO

Sarah Lewen

CHIEF EXPERIENCE OFFICER AT TALKSPACE

Led customer experience overhaul during Talkspace IPO in peak Covid.

VP MARKETING AT SONDERMIND

Owned \$35M budget, 10-person team. Series C General Catalyst-backed digital mental health platform.

EX-AMAZON

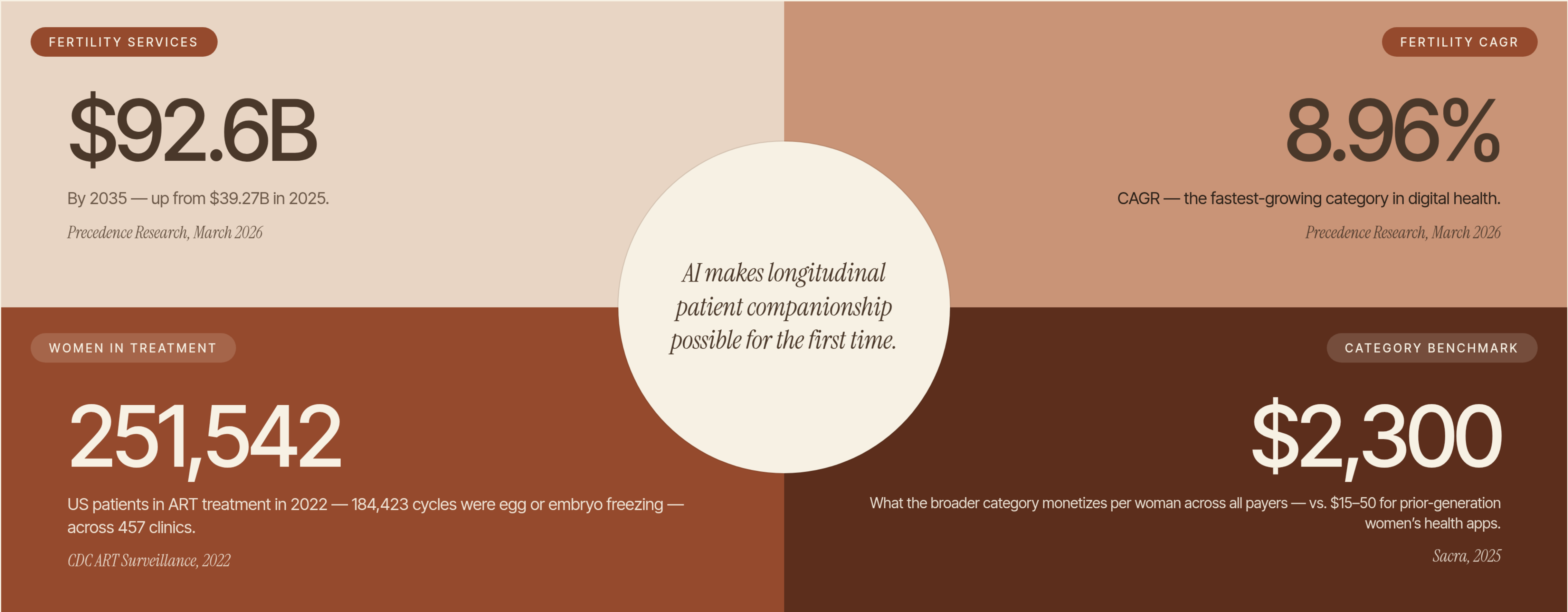
4.5 years as L6 product management & marketing leader across Amazon Fashion (B2C), Amazon Music (B2C), Amazon Advertising (B2B).

4X EGG FREEZE

Personal lived experience at CCRM Fertility and Spring Fertility.



Fertility is one of the fastest-growing segments in women’s health — projected to reach \$92.6B globally by 2035.



The clinic-led wedge is a \$2.16M business at ~30 clinics (7% penetration). The category behind it is \$92.6B.



ASSUMPTIONS & SOURCES

SOURCED ~457 US fertility clinics (CDC ART Surveillance 2022). ~400 SART-reporting clinics is the serviceable subset.

ASSUMPTION ~\$80K blended ACV = ~\$50K platform fee + per-patient usage (~400 patients × ~\$75). Platform pricing — validated in pilot.

DERIVED ~30 paying clinics × ~\$80K ACV, net of churn → ~\$2.16M ARR → ~7% of US fertility clinics.

FLOOR At point-tool pricing (~\$42K ACV), the same ramp produces ~\$1.14M. \$2.16M is the platform-pricing case, validated in pilot.

Fertility platforms are already billion-dollar businesses — and the category is still early.



\$1.7B

Employer & health-plan led · CNBC 2024



\$1.09B FY23

Public · fertility benefits · SEC filings



Series C · \$115M raised

Employer-benefits fertility · Crunchbase

PROBLEM

Fertility medicine works.
The experience around it is broken.
Carousel fixes that.

The key unlock no one has solved: the patient experience itself.

Verbatim quotes from 30 patient interviews across NYU Langone, Weill Cornell, Spring Fertility, CCRM, Kindbody, Shady Grove & RMA — Q1 2026.

LONELY

“It’s the most lonely thing I’ve ever done in my entire life. You don’t get it unless you’ve been through it.”

KATIE · 35 · IVF & SURROGACY

MISINFORMED

“I wish I had known how many eggs I’d have at 30 versus 35. I could have done one round at 30 and gotten all the eggs I’d ever need.”

MAIA · 39 · EGG FREEZING

INCONSISTENT

“Is there one person who’s looking at me from start to finish?”

NOUR · 39 · EGG FREEZING

Poor patient experience is a measurable revenue leak for clinics.

~500

cycles per year at a mid-size clinic



\$22K

all-in revenue per IVF cycle, higher-cost markets

Industry cost surveys, 2024–2025



~25%

drop out after their first cycle

Multiple peer-reviewed cohorts, 2008–2024

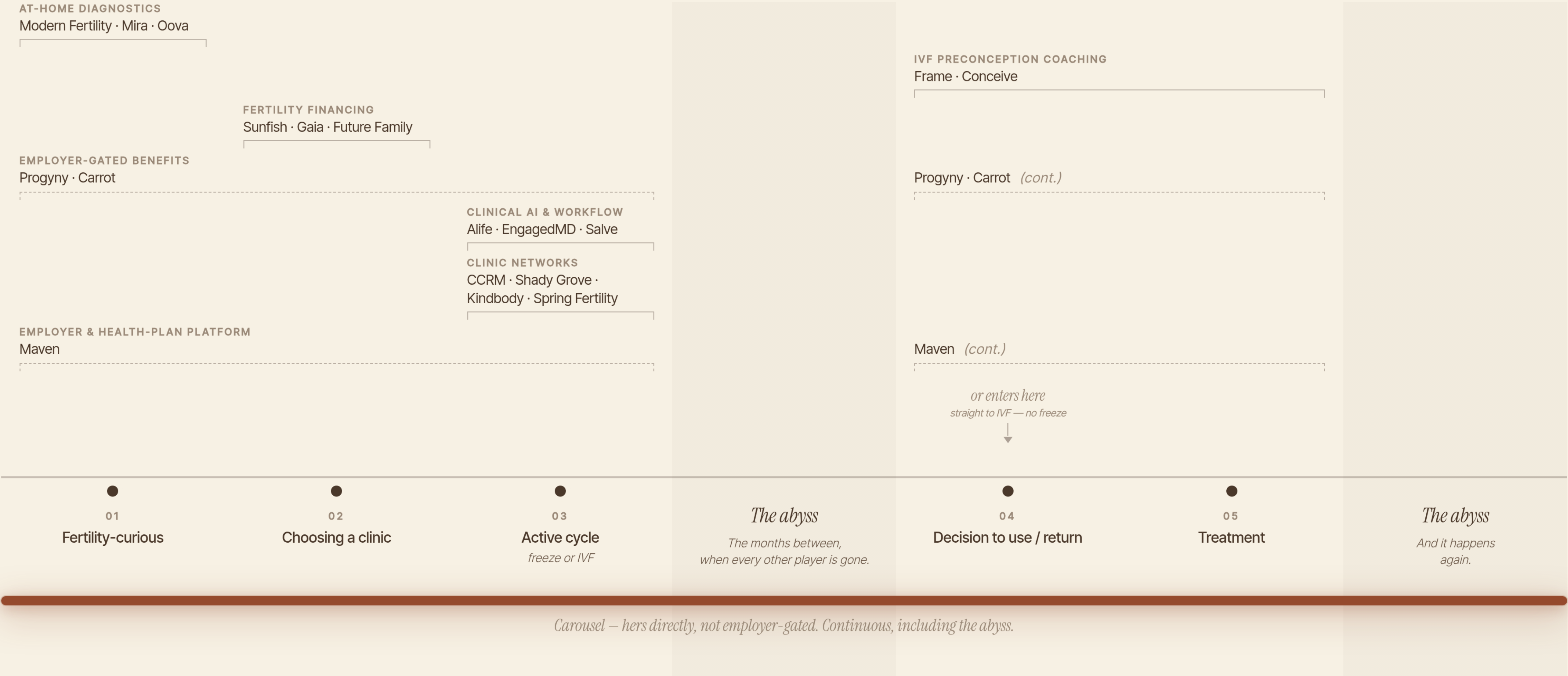


~\$1.69M

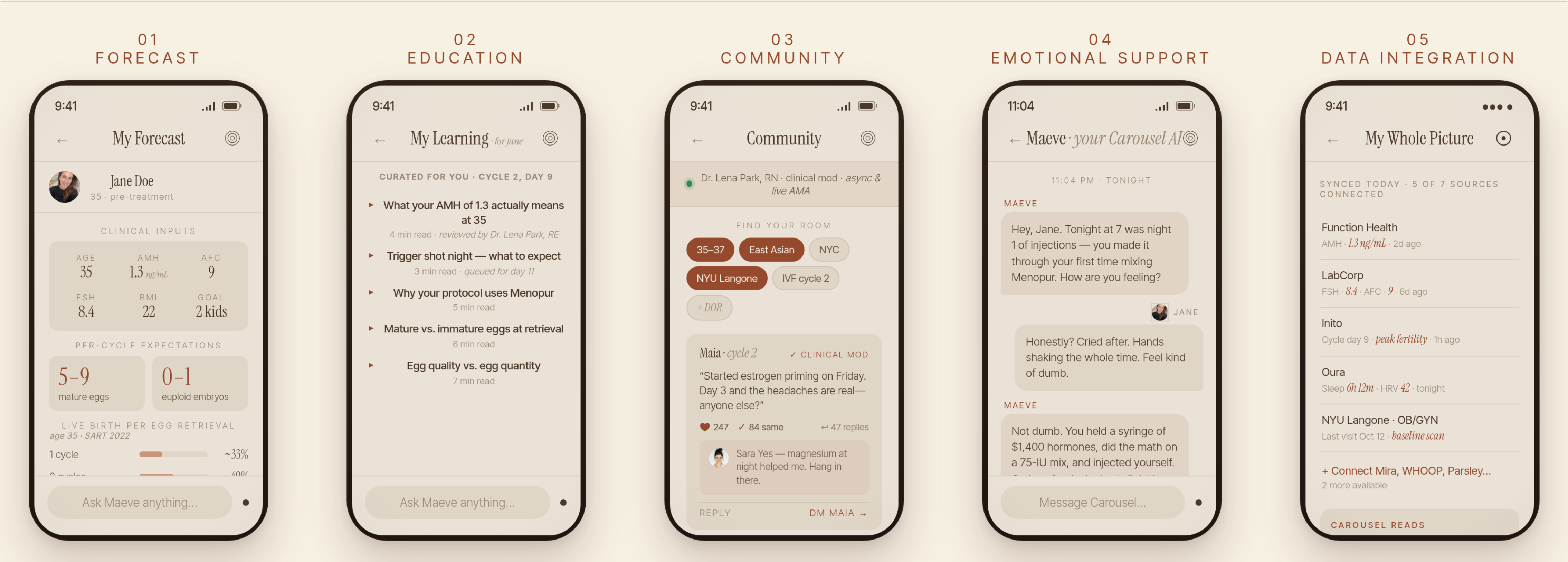
annual revenue exposure per clinic

$500 \times 25\% \times \$13,500$

Others span the journey too — but theirs is employer-gated. Carousel is *hers*, directly, and stays.

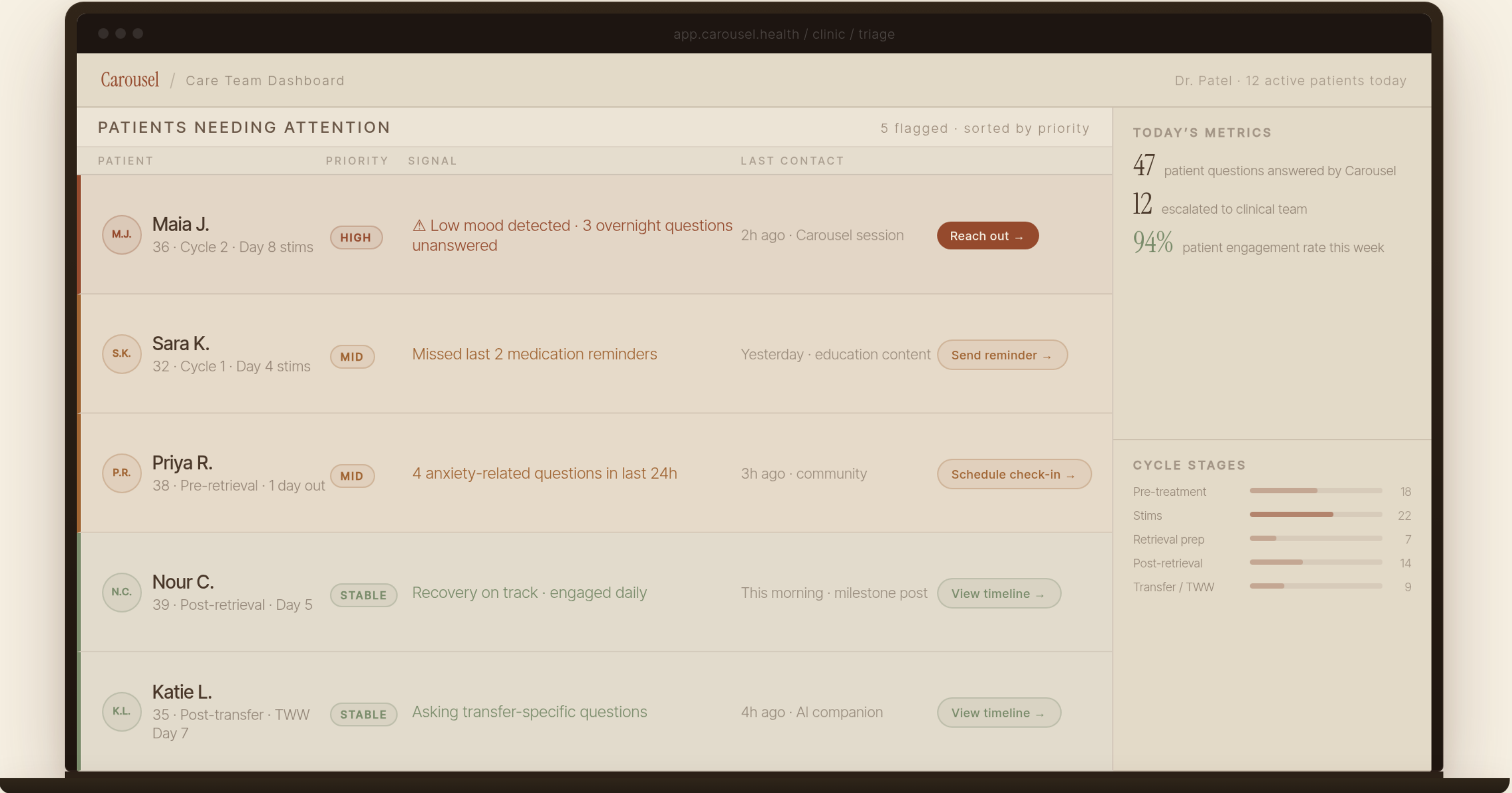


Carousel is the AI-first, digitally-native, longitudinal patient experience platform for women on their fertility journey.



For clinics: a real-time view of every patient’s journey — and where she needs support.

IN DEVELOPMENT WITH CLINICAL INPUT. THE PATIENT EXPERIENCE PLATFORM SHIPS FIRST.



01 Patient triage view

See who needs support before they have to call. Carousel surfaces patients at risk of cycle dropout, emotional distress, or protocol confusion — in real time.

02 Cycle-synced visibility

Every patient, every stage, surfaced live. Care teams see exactly where each woman is in her protocol — and what she’s been asking Carousel about.

03 Communication burden, lifted

Carousel handles the 11 PM questions so coordinators don’t have to. Routine reminders, milestone check-ins, and triage — automated and personalized.

One product. The woman’s experience is the same regardless of who pays.

NOW

Clinic

The clinic adopts Carousel because reducing dropout protects cycle revenue. We sell the ROI, not the software.
Revenue-share or white-label, where the clinic gains a line of business — is upside we’ll model with pilot data.

WHY FIRST

It’s where the money problem is. The wedge that makes everything else credible.

LATER

Employer

Maven and Carrot proved employers pay for fertility benefits. That lane is occupied and the sales motion is 12+ months out.

WHY LATER

Validated demand, wrong time. We need the clinic proof point first.

LATEST

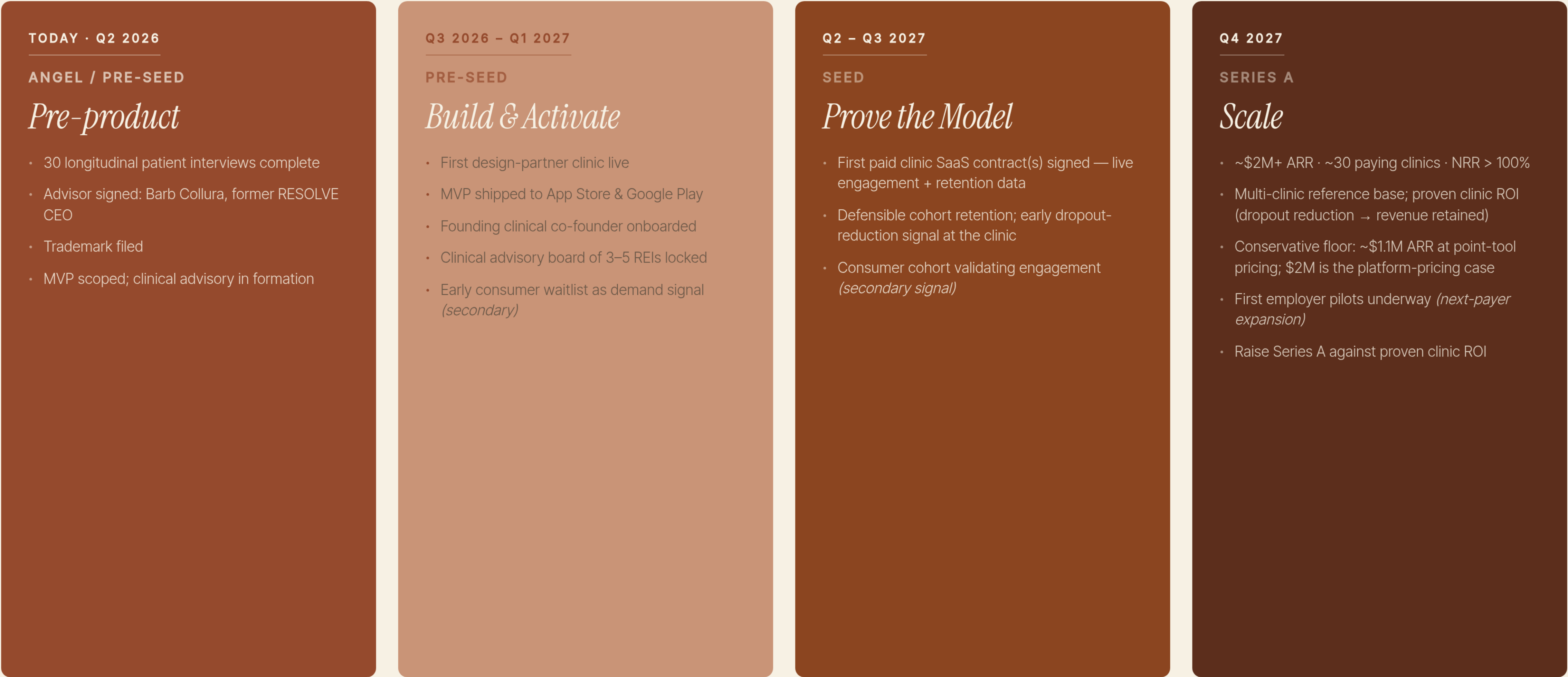
Health Plan • System • Medicaid

Real at scale. Year-plus motion. Requires everything above to be true first.

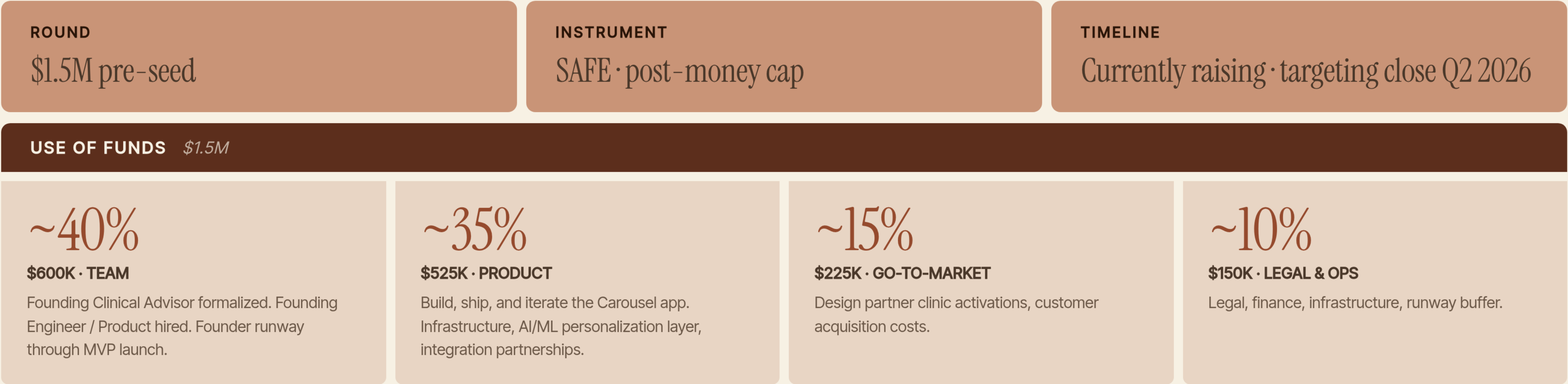
WHY LATEST

Largest pool, slowest buyer. Built for after we’ve earned the right to be in the room.

From thirty interviews to a clinic-proven model.



Carousel is opening the pre-seed — \$1.5M on a SAFE.



Let's talk.
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